

Results

Content Validity

The minimum **I-CVI** and the computed **S-CVI** for the study module were 0.78 and

0.93, respectively (Table 2). The minimum value of I-CVI was given to items 5, 10 and 11 (0.78) and Items 1-4 and 12-16 had the complete value (1.00) (Table 2).

Table 2. The ranking of the questionnaire's items according to the experts (n=9)

Item	Number of Agreement (ranked 3 or 4)	I-CVI
1	9	1.00
2	9	1.00
3	9	1.00
4	9	1.00
5	7	0.78
6	8	0.89
7	8	0.89
8	8	0.89
9	8	0.89
10	7	0.78
11	7	0.78
12	8	1.00
13	9	1.00
14	9	1.00
15	9	1.00
16	9	1.00

S-CVI (Based on mean):
0.93

Therefore, the results of the assessments showed that the majority of the experts believed that the module was valid according to the study population and objectives. However, the essential changes were applied based on the experts' recommendation such as improving the activities details and steps, adding picture to the module as well as highlighting the target life skill(s) in each session.

Face Validity

Most of the participants in both groups were female (19 females vs. 11 males) with Malay ethnicity (14 Malay vs. 7 Chinese and 9 Indian). Although the participants did not live with their parents, 40.2% of them had at least one living parent.

Using Paired Samples t-Test, the results showed that the mean score of 6 coping methods including active coping ($t=-5.85$, $P<0.001$), using of emotional supports ($t=-4.80$, $P<0.001$), using of instrumental support ($t=-4.89$, $P<0.001$), positive reframing ($t=-3.89$, $P<0.001$), planning ($t=-4.91$, $P<0.001$) and acceptance ($t=-5.98$, $P<0.001$) methods significantly increased after interventional sessions. Meanwhile, the mean score of 3 coping methods including self-distraction ($t=5.43$, $P<0.001$), denial ($t = 2.80$, $P<0.02$) and self-blame ($t=7.24$, $P < 0.001$) significantly decreased in post-test. No significant change was found in mean score of substance use, behavior disengagement, venting, humor and religion ($p>0.05$).